



UNIT PENGURUSAN KUALITI
HOSPITAL TUNKU AZIZAH
(HOSPITAL WANITA DAN KANAK-KANAK)
ARAS 6, PEJABAT PENTADBIRAN
JALAN RAJA MUDA ABDUL AZIZ, KAMPUNG BARU
50300 KUALA LUMPUR

NO. TELEFON: 03-2600 3000
NO. FAKS : 03-2600 3476

Rujukan Kami : HWKKKL.100-8/2/25 (9)

Tarikh : 18 Mac 2022

SEPERTI SENARAI EDARAN

YBhg. Datuk/ Datin/ Tuan/ Puan,

EDARAN *ANALGESIC LADDER FOR ACUTE PAIN MANAGEMENT* DAN SKALA KESAKITAN DI HOSPITAL TUNKU AZIZAH (HTA)

Dengan segala hormatnya merujuk kepada perkara di atas.

2. Untuk makluman YBhg. Datuk/ Datin/ Tuan/ Puan, Jawatankuasa Program Bebas Kesakitan HTA telah menyediakan dokumen *Analgesic Ladder for Acute Pain Management* dan Skala Kesakitan seperti di **Lampiran 1** dan **Lampiran 2** untuk kegunaan di wad/ klinik/ Jabatan. Unit Pengurusan Kualiti (UPK) HTA akan mengedarkan dokumen tersebut kepada wakil dari setiap wad/ klinik/ Jabatan pihak YBhg. Datuk/ Datin/ Tuan/ Puan.

3. Sehubungan dengan itu, mohon kerjasama pihak YBhg. Datuk/ Datin/ Tuan/ Puan untuk memastikan dokumen tersebut dipaparkan untuk kegunaan di wad/ klinik/ Jabatan masing-masing.

Perhatian pihak YBhg. Datuk/ Datin/ Tuan/ Puan dalam perkara ini amat dihargai.

Sekian, terima kasih.

“WAWASAN KEMAKMURAN BERSAMA 2030”

“BERKHIDMAT UNTUK NEGARA”

Saya yang menjalankan amanah,

DR SHAMSUL ANUAR BIN KAMARUDIN
MMC : 35873
PENGARAH
HOSPITAL TUNKU AZIZAH
(HOSPITAL WANITA DAN KANAK-KANAK)

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(DR. SHAMSUL ANUAR BIN KAMARUDIN) MMC: 35873

Pengarah
Hospital Tunku Azizah
CSM/PAIN/2022

Rujukan kami: HWKKKL.100-8/2/25 (q)

s.k.:

- i. Timbalan Pengarah (Perubatan) I**
Merangkap Pengerusi Jawatankuasa Program Bebas Kesakitan HTA
- ii. Timbalan Pengarah (Perubatan) II**
- iii. Timbalan Pengerusi**
Jawatankuasa Program Bebas Kesakitan HTA

SENARAI EDARAN

1. **Datuk Dr. Alzamani bin Mohammad Idrose**
Ketua Jabatan Kecemasan dan Trauma
2. **Dr. Mohd Yusof bin Abdullah**
Ketua Jabatan Pembedahan Pediatrik
3. **Dr. Thohiroh binti A. Razak**
Ketua Jabatan Anestesiologi dan Rawatan Rapi
4. **Dr. Sabeera Begum binti Kader Ibrahim**
Ketua Jabatan Pediatrik
5. **Dr. Shahril bin Abu Bakar**
Ketua Jabatan Obstetrik dan Ginekologi
6. **Dr. Nor' Ashikin binti Johari**
Ketua Jabatan Orthopedik Pediatrik
7. **Dr. Shifa binti Zulkifli**
Ketua Jabatan Otorinolaringologi
8. **Dr. Norhafizah binti Hamzah**
Ketua Jabatan Oftalmologi
9. **Dr. Sa'adah binti Atan**
Ketua Jabatan Pergigian Pediatrik
10. **Dr. Ramli bin Mohd Ali**
Ketua Jabatan Psikiatri dan Kesihatan Mental
11. **Dr. Azmi bin Alias**
Ketua Jabatan Pembedahan Neurologi
12. **Encik Mohammad Nazli bin Mohamed Khalid**
Ketua Unit Penyeliaan Hospital
13. **PJ Norziyatul Akmar binti Mohamad Idris**
Ketua Unit Kejururawatan



PAIN FREE PROGRAM
Transformasi Konsep Rawatan
Pelanggan Bebas Kesakitan

ALAT PENILAIAN TAHAP KESAKITAN

▶ Skala Kesakitan



SKALA KESAKITAN





0

1

2

3

4

5

6

7

8

9



10

TIADA KESAKITAN

Adopted from IASP 2017

KESAKITAN YANG AMAT SANGAT

▶ Skala Kesakitan (Muka)



SKALA KESAKITAN (MUKA)





0

2

4

6

8



10

TIADA KESAKITAN

Adopted from IASP 2017

KESAKITAN YANG AMAT SANGAT

* Bagi kegunaan pesakit berumur 3 - 7 tahun & pesakit yang tidak dapat menyatakan tahap kesakitan

▶ Skala FLACC



Skala Pemarkahan Kesakitan Melalui Pemerhatian Tingkah Laku

Kategori	Permarkahan		
	0	1	2
Wajah	Tiada ekspresi tertentu di wajah atau dalam keadaan tersenyum	Kadang terlihat muka berkerut, murung, tidak bermaya atau tidak bersemangat	Rahang terkancing, dagu bergetar (pada kadar kerap hingga berterusan)
Kaki	Kedudukan biasa atau selesa	Keadaan tidak selesa, resah atau tegang	Menendang-nendang atau membengkokkan kaki
Aktiviti	Berbaring tenang, berkedudukan biasa, bergerak dengan selesa	Berguling, berganjak depan dan belakang, tegang	Meringkuk, kaku atau menggelupur
Tangis	Tidak menangis (keadaan tidur atau terjaga)	Merengek dan kadang berpeluh	Menangis berterusan, berteriak dan teresak-esak, sering mengeluh
Kebolehpujukan	Tenang	Masih dapat dipujuk dengan sekeali sentuhan, pelukan atau kata-kata sehingga mudah terganggu	Sukar dipujuk

Setiap kategori diberi markah 0-2 dengan jumlah keseluruhan 0-10

MAKLUMAT LANJUT

PORTAL MYHEALTH
• www.infohiat.gov.my
• www.myhealth.gov.my



Program Bebas Kesakitan myhealthkkm



Diorbitkan oleh:
Bahagian Pendidikan Kesihatan
Kementerian Kesihatan Malaysia
2018

ANALGESIC LADDER FOR ACUTE PAIN MANAGEMENT				SEVERE Score 7-10		UNCONTROLLED
MILD Score 1-3		MODERATE Score 4-6		Regular Morphine or IR Oxycodone ± PCM ± NSAIDs/ COX 2 inhibitor	PRN Additional Morphine or IR Oxycodone	
Regular	PRN	Regular	PRN			
No medication or PCM	PCM &/ or NSAIDs/ COX 2 Inhibitor	Tramadol or Morphine ± PCM ± NSAIDs/ COX2 inhibitor	Additional Tramadol or Morphine			To refer to APS for: PCA or Epidural or other forms of analgesia

RECOMMENDED DRUG DOSAGES FOR ACUTE PAIN MANAGEMENT IN CHILDREN

(Review all regular use of analgesics no later than 72 hours after initiation)

Drug	Route	1 month-2 years	2-12 years	12- 18 years	> 18 years
		Dose and frequency			
Paracetamol	Oral	0- 3 months: 15mg/kg 6 – 8 H (Max: 60mg/kg/day) > 3months -12 years: 15mg/kg 4-6 H (Max: 90mg/kg/day or 4 grams/day)		500 mg - 1 gram 4-6 H (If non- obese ≥ 50 kg: 1 gram 4-6 H) (Max: 4 grams / day)	500 mg – 1gram 4-6 H (Max: 4 grams/day)
	Per rectal	0- 3 months: LD 30 mg/kg; MD : 20mg/kg 8 H (Max: 60mg/kg/day) > 3 month – 12 years : LD 40mg/kg ; MD : 15-20mg/kg 6 H (Max: 75mg/kg/day)			
	IV	Preterm neonate over 32/52 (CGA) : 7.5mg/kg 8 H Term neonate & until 10 kg: 7.5 mg/kg 6-8 H(Max: 30mg/kg/day) > 10kg or child up to 50 kg: 15mg/kg 4- 6H (Max: 60 mg/kg/day, not exceeding 2 grams if < 33 kg, or 3 grams for 33-50 kg.)		If non obese > 50 kg : 1 gram 4 – 6 H (Max: 4grams/day) **Obese Children: 15mg/kg Adjusted body weight (Max: 4grams/day)	
Ibuprofen (NSAIDS)	Oral	< 3 months: not recommended > 3 months: 5mg/kg 6-8 H (Max: 20mg/kg/day) 6 months – 12 years: 5-10 mg/kg 6-8 H (Max: 30-40mg/kg/day or 1.2 grams/24 hours)		200 mg – 400 mg 4 - 6H (Max: 2.4 grams/day)	
Diclofenac (NSAIDS)	Oral	< 6 months: not recommended > 6 months or >10 kg: 0.3-1mg/kg 8 H (Max: 3 mg/kg/day up to 150 mg/day, whichever is less for 2 days)		Oral 25 - 50mg 8 H (Max: 3 doses/day)	
	Per rectal	> 1 year: 1 mg/kg 8-12 H (Max: 3 mg/kg/day up to 150 mg/day, whichever is less)		50-100 mg (oral to be started 18 hours after initial 100mg suppository)	
Mefenamic acid (NSAIDS)	Oral				> 14 years: 250mg 8 H 250-500mg 8 H
Celebrex (Cox2 inhibitor)	Oral	< 2 years: not recommended		> 2 years: weigh risks & benefits 10-25 kg: 50mg 12 H > 25 kg: 100mg 12 H	Celecoxib: 200mg 12H / 400mg daily (Max for 1 week)
Tramadol	Oral / IV	> 1 year: 0.5-1mg/kg 4-6 H (with caution) NB: For tonsillectomy max 1mg/kg /dose 6-8H		> 12years: 1mg/kg 4 - 6H (Max: 100mg/dose or 400mg/day) 50mg -100mg 6-8 H (Max 400mg/day)	
Oxycodone	Oral	> 1 month: Immediate Release (IR Oxynorm) 0.1-0.2 mg/kg (max5mg) PRN or 4-6H (by APS / Palliative team) NB: Immediate release (IR Oxynorm) for acute pain; Slow release (SR Oxycontin) for severe background pain			IR Oxynorm 5-10mg 6 H SR Oxycontin 10 -20 mg BD
Morphine (Opioid μ -agonist)	Oral	> 1 month-1 year: 0.1mg/kg 4-6 H (for moderate – severe pain) (with caution) > 1 year: 0.1-0.2 mg/kg 4-6 H (for moderate pain) 0.2-0.4 mg/kg 4-6 H (for severe pain)		> 12yrs: 10-15mg (up to 6x/ 24 hours)	
	SC	0.1-0.2 mg/kg < 6 months: (up to 4x/ 24 hours) > 6 months: (up to 6x/ 24 hours)	0.2 mg/kg (up to 6x/ 24 hours)	> 12yrs: 5 -10mg (up to 6x/ 24 hours)	
	IV	Slow titration : Refer iv morphine titration protocol 1-12 months: Max : 0.1 mg/kg (up to 4x /24 hours) > 1 year: Max : 0.1 mg/kg (up to 6x/ 24 hours)		Slow titration : Refer iv morphine titration protocol > 12yrs: 2.5 -10mg (up to 6x/ 24 hours)	
Naloxone(pure opioid antagonist)	IV	0.01 mg/kg IV (Max: 0.4 mg) may repeat every 2 minutes.			0.1 – 0.4 mg IV/IM/SC IV dose may repeat every 1-2 minutes
Ketamine	Oral/ IV	Oral: 2-10mg/kg; IV: 0.2-0.5 mg/kg (restricted use only for trained personnel) (To be used in combination with midazolam for procedural analgesia if Ketamine dose > 0.5 mg/kg)			

IV : intravenous, SC : subcutaneous, H : hourly, Max : maximum, LD : Loading dose, MD: Maintenance dose, IBW : Ideal body weight.

**For Obese Children, recommended adjustments for drug dosing:

Opioid: Ideal Body weight (IBW); Paracetamol and NSAID: Adjusted Body Weight= IBW+ 0.4 x (Actual BW-IBW)